



Lyfe Place Abuja

The address, not the practice.

Commercial structure, tenancy model and investment case
The Cloister by Lyfe Place / Alameda specialist clinic / campus capture

Prepared for Dr Itunu Akinware, Group CEO, Medbury Healthcare Group

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SUPERSEDED IN PART. This was the first full note and it predates several decisions. Still current: the area reconciliation, the joint venture amendments, the conditions precedent and the regulatory pathway. **Superseded:** the financial model, the space plan, the operating hours and the tenant mix. Mezo Health now manages the campus on a fee, the first floor yields eight consulting rooms rather than ten, the operating window is 08:00 to 22:00, a day case theatre is proposed as a ground-level unit, and Medlyfe is out of phase one. For current figures use the engagement note, and for the space plan use the allocation drawing.

01 / THE RECOMMENDATION

Medbury is the landlord, not the practice.

Medbury takes the Abuja property as head lessee and campus operator, and lets every clinical line to a tenant. No consultants on Medbury's payroll, no imaging equipment on Medbury's balance sheet, and no clinical staffing risk. Medbury earns from three layers, in ascending order of value.

Revenue layer	What it is	NGN M / yr
1. Fixed leases	Alameda and Medlyfe. Pharmacy and diagnostics are facility, not tenants	168.5
2. Sessional	10 consulting rooms and 1 procedure room, sold by the block, 07:00 to 21:00 Mon to Sat	330.9
3. Capture	Diagnostics and pharmacy margin on roughly 22,700 patient contacts a year	210.0

The single most important number in this note. Two fixed leases produce NGN 168.5M a year against a fixed cost base of NGN 187M. **The campus breaks even at under 6% sessional fill,** about five blocks a week across ten rooms. Everything the sessional programme and the capture layer produce beyond that is margin.

That reframes the risk correctly. This is not an operating risk, it is a **leasing and lease-term risk**. The question is not whether the campus can trade profitably. It is whether the leases get signed and whether the head lease can be extended from two years to five plus five before fit-out capital is committed.

Three corrections to what is currently in view

Item	Currently assumed	Corrected
Building area	1,235 sqm	~711 sqm gross, ~520 sqm net lettable
Total project capital	~NGN 185M	~NGN 527M on a light-touch fit-out
Head lease term	2 years	Must be 5 plus 5 before fit-out capital is released

02 / THE MODEL

The address, not the practice.

Lyfe Place Abuja is a physicians' address. It is the Harley Street model already worked up for Ikoyi, on the same asset class and the same lease quantum, with three advantages Ikoyi does not have: a contracted anchor user from day one, two Medbury-owned support businesses on site, and imaging capital carried by a partner rather than by Medbury.

The tenant classes

Class	Who	What they buy	Instrument
Users	Alameda, Doctors Foundation for Care, private consultants, Cloister physicians	Access to patients and an address	Session tariff or blended licence
Facility	Medbury Diagnostics (lab and imaging), Medbury Pharmaceuticals	Sold as part of the campus offer, not billed as tenants	Recovered in tenant rates
Wellness	Medlyfe longevity and infusion	A discrete address in the guest chalet	Base rent

Users are priced to **fill**, not to maximise rent per sqm. An empty session is not just lost rent, it is a lost diagnostic basket and a lost prescription. The tenant mix exists to drive patient contacts through the door, because that is where the return is.

Two structural decisions

Radiology sits under Medbury Diagnostics. Medbury Diagnostics takes the whole diagnostics demise, laboratory and imaging together, and contracts the radiology partner itself. The landlord has one diagnostics counterparty and never has to police who owns ultrasound. The partner's equipment, shielding and room fit-out are Medbury Diagnostics' commercial arrangement, not a lease term.

Pharmacy and diagnostics are facility, not tenants. Their space is sold as part of the campus offer and recovered through the rates the users pay. That is a better pitch: on-site laboratory, imaging and pharmacy is a real reason a consultant picks Lyfe Place over a room elsewhere, and it turns the first-call obligation from a restriction into a benefit. Same-visit bloods and scripts, included. Charge the NGN 54.1M internally in the management accounts anyway, so the numbers stay arm's length for FIRS and for any future investor in PropCo, and so diagnostics has to justify its 115 sqm.

The Cloister is a membership, not a room. No dedicated family-practice suite. Its physicians buy blocks from the sessional pool like every other user. That avoids a fit-out commitment against an unproven Abuja panel, lets the membership launch before the first floor is finished, and keeps the rule that everybody is a tenant.

The Cloister membership is also the funding mechanism. Memberships are collected annually in advance and the head rent is paid in advance. Sixty founding Abuja memberships at a blended NGN 2.2M is NGN 132M, which covers the NGN 115M of rent and fees outright. Presell the membership, then pay the landlord.

03 / THE BUILDING

It is roughly half the size the tenant-mix deck assumes.

Measured from the as-built drawings for the main house, guest chalet and boys' quarters. This needs verifying against the CAD before it goes any further, but if it holds it re-bases every number in the current plan.

Structure	Deck assumes (sqm)	As built (sqm gross)
Main house, ground floor	440	286
Main house, first floor	440	277
Guest chalet	160	96
Boys' quarters	55	52
Total gross	1,235	711

Net internal at 87% is about 619 sqm. Net lettable, after reception, waiting, corridors, stairs, plant and shared toilets in a converted residential house, is about **520 sqm**. The deck overstates the building by 74%.

The tell: the deck's service charge of NGN 60,000 to NGN 85,000 per sqm. Divide the real annual operating cost by the phantom 1,235 sqm and you get NGN 61,000. Divide it by the real 520 and you get a materially higher number. The service charge was back-solved from an area that does not exist.

Three consequences

- The seven-unit tenant mix does not fit. Something has to come out.
- Alameda's two consulting rooms and Dr John's 440 sqm specialist centre describe two different buildings, and neither of them is this one.
- Every per sqm figure in the deck needs re-basing on 520 sqm.

04 / CAPITAL

What it actually costs.

Item	NGN M
Head rent, 2 years paid in advance	100.0
Agency, legal and caution deposit at 15%	15.0
PropCo fit-out	232.0
Power system, 25KVA hybrid solar and lithium	33.0
Medbury Diagnostics unit fit-out	28.0
Medbury Pharmaceuticals unit fit-out	24.0
Working capital, six months	95.0
Total Medbury cash	527.0

Against roughly NGN 185M currently in view. On a sound building this is not a conversion, it is decoration, partitioning, cooling, ICT, fire systems and FF&E. The building fit-out element is NGN 148M over 711 sqm, about NGN 208,000 per sqm, which is mid-range for a light-touch clinical conversion in Nigeria. Six packages account for half of it and would cost the same in a brand new house: cooling at NGN 24M for about 28 units, ICT at NGN 15M, fire at NGN 9M, partitioning at NGN 8M, mechanical fresh air at NGN 6M and medical waste at NGN 3M. Full package-by-package breakdown, with a full-conversion and a lean case alongside, is in the companion fit-out note.

Phasing, if capital needs staging

Phase	Scope	NGN M
Phase 1, to open	Ground floor complete, reception, 6 sessional rooms, all services, fire, cooling, water, power	~400
Phase 2, months 7 to 12	Remaining 4 sessional rooms, procedure room, doctors' lounge. Funded from trading	~85
Phase 3, year 2	Guest chalet, boys' quarters back of house, external works and signage	~42

At NGN 232M the case for phasing largely disappears, and doing it in one pass avoids the cost and disruption of returning contractors to a trading clinical building. Phasing remains the fallback if capital has to be staged.

Do not commit fit-out capital against a two-year head lease. NGN 284M of fit-out over two years is NGN 142M a year of amortisation, which takes the cost base to about NGN 289M against fixed leases of NGN 168.5M and pushes break-even to 36% sessional fill. Achievable, but it removes the entire margin of safety. Over a five plus five it is NGN 28M a year, which the campus carries comfortably.

05 / POWER AND HOURS

Longer hours are the whole business case.

Nothing in scope needs central plant. Consultations, screening, imaging, point-of-care laboratory, infusions and minor procedures under local anaesthetic are comfort-cooling loads, served by zoned inverter split units. Central air handling, pressure cascade and filtration would only be justified by an operating theatre, day surgery, an isolation room or a compounding cleanroom, none of which are in scope. If Medlyfe compounds infusions, the answer is a laminar flow cabinet at NGN 4M to NGN 8M, not a cleanroom.

The operating window

At the agreed 15,000 to 18,000 kWh a month, the campus supports **07:00 to 21:00, Monday to Saturday**. Against a 9 to 5 weekday operation that is 40 sellable hours per room per week going to 84, a 110% increase in capacity from the same building.

Cost line	9 to 5 (NGN M)	Extended (NGN M)	Delta
Power	18	28	+10
Admin and facilities salaries, shift cover	15	33	+18
Security, cleaning, waste, insurance, internet	12	15	+3
Maintenance and biomedical	6	8	+2
Campus marketing and concierge	6	8	+2
Incremental cost of extended hours			+35

NGN 35M of cost buys roughly NGN 250M of revenue. But the stronger argument is supply, not demand. Most Abuja specialists hold full-time posts at the federal, teaching and military hospitals. Evenings and Saturdays are the only windows in which they can do private practice at all. A 9 to 5 address can only recruit consultants already in full-time private practice, which in Abuja is a small pool. A 07:00 to 21:00 six-day address can recruit the city.

It also fixes the throughput problem. The same patient volume spread across fourteen hours instead of eight is a comfortable arrival rate rather than a crowded waiting room, which is the difference between a premium address and an ordinary one.

Four conditions, or the extra hours do not pay. Medbury Diagnostics and Pharmaceuticals must staff the full window, or every evening consult loses its basket. Size the battery, not the array, because evenings cannot run on solar. Security and lit parking to 21:00. And halve the service intervals on the split units, since fourteen hours over six days is double the duty cycle they are specified for.

06 / THE TENANCY STACK

Who sits where, and what they pay.

Space plan on the real 520 sqm net lettable

Floor	Use	sqm
Ground	Medbury Pharmaceuticals	40
Ground	Medbury Diagnostics, laboratory and imaging	115
Ground	Alameda priority rooms, 2	28
Ground	MDT and telemedicine, bookable	18
First	Sessional consulting rooms, 10 at 14 sqm	140
First	Procedure room	22
First	Doctors' lounge and touchdown, common	27
Chalet	Medlyfe longevity and infusion, whole building	80
BQ	Back of house: staff, records, medical waste holding, plant	45

Imaging must be on the ground floor. Heavy equipment, lead shielding, no lift. The boys' quarters goes to back of house rather than an office let, because a fourteen-hour six-day operation needs staff facilities and a compliant medical waste route and the main house has nowhere to put either.

Rate card

Sessional tariff	Block	NGN per block
Sessional consulting, weekday early 07:00 to 09:00	2 hours	44,000
Sessional consulting, weekday daytime	4 hours	61,000
Sessional consulting, weekday evening 17:00 to 21:00	4 hours	94,000
Sessional consulting, Saturday	3.3 hours	83,000
Procedure room	per session	248,000
Guest and ad hoc use	any band	about 2x member rate

Fixed lease	sqm	NGN / sqm / yr	NGN M / yr
Medlyfe, guest chalet	80	456,000	36.5
Alameda, blended licence	see 07	blended	132.0
Medbury Diagnostics, laboratory and imaging	115	facility	nil
Medbury Pharmaceuticals	40	facility	nil
Total fixed leases			168.5

Rates carry an uplift of about 11% over a bare-room tariff, which is what the on-site laboratory, imaging and pharmacy are worth to a consultant running a private list. Evening and Saturday carry a further premium because they are the hours nobody else in Abuja offers. Weekday daytime is discounted deliberately, because it is the weakest band and an empty room earns nothing. With pharmacy and diagnostics treated as facility, **56% of the building is facility and 44% is let**, which is what a fully serviced campus looks like and why the rate per let sqm has to be high.

07 / ALAMEDA

One blended charge, and a hard capacity constraint.

Alameda brings 15 to 20 Egyptian specialists to Nigeria in rotating conversion drives, not as a standing team. Their access window is 09:00 to 17:00, Monday to Friday, non-exclusive, to two priority consulting rooms with additional rooms and the procedure room available during declared drive windows.

What actually caps their volume

Four independent limits. Real throughput is the minimum of the four.

Limit	Basis	Consults per day
1. Room capacity	Rooms x 7 productive hours x 2.5 consults per hour x 80% utilisation	2 rooms = 28 4 rooms = 56
2. Clinician capacity	Specialists x 14 to 16 consults a day, sustained over a drive	2 = 28 4 = 56 6 = 84
3. Campus throughput	8 arrivals an hour in a VIP setting x 7 hours	56 a day
4. Booked demand	Appointments generated and shown	20 to 30 early, 50 to 56 mature

Limits 1 and 2 must match one to one. Four specialists require four rooms. Fifty-six consultations cannot be delivered through two rooms; two rooms cap at 28. The design point is four rooms and four specialists at 56 a day, which is also exactly where reception capacity caps out. Do not allocate a fifth room. It only adds arrivals the building cannot absorb.

Drive model	Low	Base	High
Drives per year	4	6	8
Clinic days per drive	6	8	10
Specialists, and therefore rooms	2	4	4
Consults per day	28	56	56
Consults per year	672	2,688	4,480

The charge

Component	NGN M / yr	USD / yr
Rent, priority rooms and MDT room	38.75	25,000
Facility and services, laboratory, imaging and pharmacy included	93.25	60,000
Blended total	132.00	85,000

One number. No separate drive fee, no mobilisation line, no marketing line, no throughput charge. Marketing sits inside the service charge undifferentiated and Medbury simply markets. Payable **twelve months in advance in USD**, escalating 12% a year. NGN 132M is the ask and **NGN 114M the floor**, since Alameda have already named a USD 25,000 property budget and the uplift for facility recovery is the part they will resist.

The rent line takes Alameda's entire stated USD 25,000 property budget, and is set at market for the demise so it survives an arm's length test under clause 13 of the joint venture agreement and under Nigeria's transfer pricing regulations. The service charge is a separate cost centre, so "the budget is spent" is not available to them.

One structural note for the model. The charge is billed to the JV OpCo, of which Medbury owns 49%. The net economic transfer from Alameda is therefore about NGN 67M, not NGN 132M. That is a feature of charging a company you part own, and it is the reason the capture layer matters more than the facility charge. Alameda's patients are worth more to Medbury Diagnostics than their rent is to PropCo, because capture is 100% Medbury's.

The clause that beats the rent argument. Every user agreement, Alameda's included, must carry a first-call obligation routing diagnostics to Medbury Diagnostics and prescriptions to Medbury Pharmaceuticals unless clinically unavailable. Conversion drives generate pre-operative workups, so Alameda's diagnostic attach rate is higher than any other user on the campus. There is room to move on the facility charge in exchange for binding first call. There is no room to move on first call.

08 / THE INVESTMENT CASE

Break-even before a session is sold.

Stabilised annual cost	NGN M
Head rent, escalated	55.0
Power	28.0
Admin and facilities salaries, shift cover	33.0
Security, cleaning, waste, insurance, internet	15.0
Maintenance and biomedical	8.0
Campus marketing and concierge	8.0
Fit-out and power system amortisation, 10 years	26.5
Fixed cost base	173.5

Fixed leases NGN 168.5M against a fixed cost base of NGN 173.5M. The campus breaks even at 1.5% sessional fill, under two blocks a week across ten rooms. On the tenant view of pharmacy and diagnostics it is covered outright, with NGN 25M to spare.

Sessional programme, at stabilisation

Band	Assumed fill	NGN M / yr
Weekday early, 07:00 to 09:00	35%	35.4
Weekday evening, 17:00 to 21:00	55%	118.9
Saturday	45%	51.5
Weekday daytime, non-drive days	35%	80.5
Procedure room		44.6
Total sessional		330.9

Ramp: about 30% of that in year one, 60% in year two, full in year three. Sessional fill is the single largest sensitivity in the whole model and it is unvalidated for Abuja.

Both views of pharmacy and diagnostics

Stabilised, NGN M / yr	As tenants	As facility
Alameda, blended licence	114.0	132.0
Medlyfe, guest chalet	30.4	36.5
Medbury Diagnostics, laboratory and imaging	39.1	in the rates

Medbury Pharmaceuticals	15.0	in the rates
Sessional programme	299.4	330.9
Campus revenue	497.9	499.4
Net annual contribution to Medbury	345	385

Campus revenue is the same either way. The facility view is worth about **NGN 40M** more to Medbury, and all of it comes from the sessional tariff uplift and Alameda's 51% share. Medlyfe's uplift is Medbury paying itself and nets to nothing. So the NGN 40M is conditional on the sessional users accepting about 11% and Alameda accepting about 16%. If only the sessional uplift sticks, the gain is NGN 31.5M.

Run both. The facility view is what tenants and Alameda see, and it is the better pitch. The tenant view stays in the management accounts, because it costs nothing and it is the only way to tell whether Medbury Diagnostics earns its 115 sqm, which at 30% of the building is the largest single space allocation on the campus. It also keeps related-party pricing arm's length for FIRS and leaves the numbers in the right shape for any future investor in PropCo or partner in the diagnostics business.

Three cases

Stabilised, NGN M	Downside	Base	Upside
Sessional revenue	165	331	397
Fixed leases	168.5	168.5	168.5
Less fixed cost base	(173.5)	(173.5)	(173.5)
Campus contribution	160.0	326.0	392.0
Diagnostics and pharmacy, net of staff, no rent charged	88	144	180
Less JV interest in the Alameda charge, and Medlyfe internal	(70)	(71)	(71)
Net annual contribution to Medbury	177	399	501
Return	Downside	Base	Upside
Total capital	527	527	527
Simple payback from stabilisation	3.0 years	1.3 years	1.1 years
Including a 24 month ramp	~5.0 years	~3.3 years	~3.1 years

The downside case halves the assumed sessional fill and cuts capture by 40%, and the project still returns. That is the margin of safety, and it comes from the fixed leases carrying the cost base on their own.

09 / THE JOINT VENTURE

What must change before signature.

Medbury holds 49% and two of five board seats. Alameda holds 51% and three, with simple majority voting and no defined reserved matters. Meanwhile Medbury funds the property outside the venture entirely. The joint venture's total initial capital of USD 152,000 is about NGN 236M against a project cost of NGN 527M, so the venture is not funding this campus. Medbury is.

The structure we have built resolves that correctly: the property stays in a wholly owned Medbury PropCo and the joint venture is a tenant paying market rate rent. On that basis 49% is tolerable. What is not tolerable is the drafting.

Priority amendments

Amendment		Why
1	Delete clause 24.I	Termination for convenience on 30 days' notice, against a two-year prepaid rent and a NGN 284M fit-out, is the most dangerous line in the document. Replace with no termination for convenience before year five, then twelve months' notice
2	Break fee	On early exit, unamortised fit-out plus remaining head rent. Today there is no route for Medbury to recover a naira of the fit-out
3	Define the reserved matters	Clause 6(g) refers to "Shareholders Reserved Matters" and never defines them. Fill the hole with a Medbury veto over budget, capex above NGN 10M, related-party contracts, borrowing, new shares, lease changes, dividend policy, MD and FD appointment, competing ventures, asset sales and winding up
4	Mutual non-solicitation, and no non-compete	A non-compete is deliberately not sought: it clashes with the independent specialists on the first floor and would lock Medbury out of conversion pathways with other foreign groups. What is needed instead is mutual non-solicitation of contracted physicians, members and corporate accounts, plus preferred-pathway status for Alameda in place of exclusivity
5	Deadlock mechanism	There is none. Escalation to chief executives, then a shoot-out, with an express carve-out that the property never moves
6	Currency	Capital is denominated in USD and every cost is in naira. Fix the naira equivalent or specify the CBN rate at each call, or an FX move silently rewrites who contributed what
7	Tag-along and drag-along	Absent entirely

Drafting defects for counsel

These are worth listing because of what they signal about how much care went into the document Medbury is being asked to sign.

- The special purpose vehicle is named **Allox Kings City Estates Ltd**, a real estate entity. Using a property-named shell as the healthcare joint venture vehicle invites confusion about who owns the building. Incorporate a purpose-built healthcare SPV.
- The cover page says **ALAMEDIA** Financial Limited. The body says **ALAMEDA**.
- Cites the Companies and Allied Matters Act **2004**, superseded by CAMA 2020, including the section 381 cross-reference.
- Clause 9 repeatedly refers to "this Clause 10".
- "Excluded Person" is used four times and never defined. Only "Eligible Person" is.
- Clause 8 requires pro rata funding. Clause 12 exempts shareholders from further liability. Directly contradictory.

The agreement names medical tourism conversion as the purpose of the venture and then provides no referral fee, no revenue share, no volume commitment, no transfer pricing mechanism and no audit right anywhere in its twenty-three pages. Whatever commercial position Medbury takes on that, the absence should be a deliberate decision rather than an oversight.

10 / CONDITIONS AND RISK

What has to be true before money moves.

Condition	What it is	When
Change of use approval	This is a residential building. Development Control and AMAC approval plus the landlord's written consent to change of use	Before the NGN 115M rent and fees are paid
Head lease to 5 plus 5	With the fit-out recognised and a reinstatement waiver	Before any fit-out capital is released
FCT health facility registration	Campus and each clinical tenant	Before opening
NNRA radiation licence	Required for X-ray, together with lead shielding and warning systems	Before imaging installs
MDCN temporary registration	Egyptian specialists cannot lawfully see Nigerian patients without it, and it is Medbury's facility licence at risk, not Alameda's	No certificate, no room key
NDPR cross-border transfer	Nigerian patient records reviewed by Egyptian clinicians need a data processing agreement and a lawful transfer basis	Before the first drive
Fire and medical waste	Compartmentation in a house never designed for it, and a compliant waste route	Part of Phase 1

11 / SENSE CHECK

What in this note is measured, and what is estimated.

Every figure here is either taken from a document you have supplied or built from a stated assumption. The assumptions below carry the most weight and should be tested before this model is used to commit capital.

Input	Source	Status
Head rent, fees, salaries, solar quote, Alameda's USD 25,000 budget, kWh target	Supplied by Medbury	Firm
Joint venture terms, clause references, party names, capital	Read from the executed draft	Firm
Building areas	Measured from the as-built PDFs, not the CAD	Verify against CAD. Everything re-bases if wrong
Electricity tariff	Blended NGN 250 per kWh across grid and diesel top-up	Confirm from an actual AEDC bill
Fit-out at NGN 232M	Built up by package, on the building being sound	Needs a condition survey, an electrical load assessment and a QS take-off
Sessional fill rates, 35% to 55%	Derived from the Ikoyi Harley Street model	Largest single sensitivity. Unvalidated for Abuja
Diagnostics 40% attach at NGN 40,000, pharmacy 45% at NGN 16,000	Illustrative	Validate against Medbury Diagnostics' actual data
Alameda drive volumes	Base case of 6 drives, 4 specialists, 8 days, 14 patients each	Confirm with Alameda directly

Ask Alameda for five things before the tariff is finalised: drives a year and the window for each, specialists per drive and in which specialities, target patients per specialist per day, their historical consult to travel conversion rate from any market they already run drives in, and average treatment invoice by speciality. If they will not give you the last two, that itself tells you what they expect to convert, and the charge should be priced to stand alone without any conversion upside.

12 / NEXT

Four conversations, in this order.

	Who	What
1	The landlord	Head lease to 5 plus 5, change of use consent, fit-out recognition, reinstatement waiver. Nothing else can be committed until this lands
2	Alameda	Blended licence at NGN 114M, twelve months in advance in USD, four rooms and 56 a day capped, first call on diagnostics and pharmacy, and the seven joint venture amendments
3	Medbury Diagnostics	Confirm the combined laboratory and imaging demise, and appoint the radiology partner under Medbury Diagnostics rather than as a campus tenant
4	Medlyfe	Confirm the guest chalet as a lease at NGN 36.5M a year. It is the second of the two leases that carry the campus

Both fixed leases are unsigned and Medlyfe has not yet been approached. Alongside the head lease term, that is the critical path and it is the whole of the risk. The trading model is not in doubt. The leases are.

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Prepared for Dr Itunu Akinware and Medbury Healthcare Group leadership, in confidence. Areas are measured from as-built drawings and should be confirmed against the CAD. Fit-out, tariff and attach-rate figures are estimates to be set against a quantity surveyor's take-off, a live electricity bill and Medbury Diagnostics' own trading data. Nothing here is a binding offer, and nothing here is legal or tax advice. FX reference USD/NGN 1,550.